**IN THE DISTRICT COURT OF APPEAL OF THE STATE OF FLORIDA**
**FIRST DISTRICT**

**Case No. 2025-1648**

DOROTHY D. JACKSON, as Personal Representative of the Estate of RONNIE L. NORRIS, JR.,
*Appellant,*

v.

SPECIALTY OPERATING INVESTMENT, LLC d/b/a SPECIALTY HEALTH AND REHABILITATION CENTER,
*Appellee.*

ON APPEAL FROM THE CIRCUIT COURT OF THE FIRST JUDICIAL CIRCUIT IN AND FOR ESCAMBIA COUNTY
LOWER TRIBUNAL NO. 2023 CA 002149

# ANSWER BRIEF

**Carol M. Rooney**
FL Bar No.: 72990
Hinshaw & Culbertson LLP
100 South Ashley Drive, Suite 1310
Tampa, FL 33602
Telephone: 813-868-8854
crooney@hinshawlaw.com
*Attorneys for Appellee, Specialty Operating Investment, LLC d/b/a Specialty Health And Rehabilitation Center*

**Rory Eric Jurman**
FL Bar No.: 194646
Hinshaw & Culbertson LLP
201 East Las Olas Boulevard, Suite

1450
Fort Lauderdale, FL 33301
Telephone: 954-467-7900
rjurman@hinshawlaw.com

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## INTRODUCTION

This appeal follows the trial court's striking of the opinions of a plaintiff's nursing standard-of-care expert in a Chapter 400 nursing home negligence action. The trial court, acting as a gatekeeper under Daubert, found that there were no submitted reliable principles and methods upon which the standard-of-care opinions were based, and that they were otherwise unhelpful to the jury. The plaintiff's motion to continue the trial to obtain a new expert was denied, and the nursing home's unopposed motion for summary judgment was subsequently entered.

## STATEMENT OF THE CASE AND FACTS

Jackson Files Chapter 400 Complaint Against Specialty.[1] (R. 11-17.) Jackson alleged she was the personal representative of the estate of Ronnie L. Norris, Jr., deceased. (R. 11.) The complaint alleged that Norris was a resident at a nursing home owned and operated by Specialty ("the Facility"). (R. 12.) Further, that while Norris was a resident at the Facility, "the staff and employees failed to develop an adequate care plan and properly monitor and supervise the care and treatment provided to RONNIE L. NORRIS, JR., in order to prevent him from choking to death on

[1]Appellant, Dorothy D. Jackson, as Personal Representative of the Estate of Ronnie L. Norris, Jr., will be referred to herein as "Jackson." Appellee, Specialty Operating Investment, LLC d/b/a Specialty Health and Rehabilitation Center will be referred to herein as "Specialty."

his dentures." (R. 12.) The complaint asserted a single count for "Chapter 400 Claim" against Specialty. (R. 13-17.)

Jackson Discloses Joyce Black as One of Her Two Expert Witnesses. (R. 50.) Jackson disclosed two experts: Joyce Black, PhD and Lee A. Fischer, M.D.[2] (R. 50.) Jackson described Black's proposed testimony as follows:

> Dr. Black will testify to the nurses substandard care, treatment and supervision Ronnie Norris Jr. received while at Specialty Health and Rehabilitation Center, which violated the applicable standard of care and violated Florida Statute §400 which resulted in injuries to Ronnie Norris Jr. Dr. Black will base her opinions upon her education, experience, and training, along with a review of applicable medical records and depositions. Dr. Black's testimony will be the product of reliable data, principles, and methods, all of which will be applied to the facts of this case. Dr. Black may also be called to rebut the opinions of Defendant's expert(s). Further, Dr. Black will testify about the medical bills which were incurred as a result of the Defendant's negligence and that said bills were medically necessary and reasonably related to the Defendant's negligence.
> …
> Plaintiff expects the above listed witnesses to testify concerning the substandard care and treatment rendered to Ronnie Norris Jr. during his residency at the defendant's facility as well as the damages she suffered as a result of the substandard care, treatment and supervision.
>
> (R. 68-69.)

[2]Dr. Fischer was Jackson's causation expert. (R. 69.) Specialty moved to exclude Dr. Fischer's testimony pursuant to Daubert. (R. 316.) The trial court did not rule on Specialty's motion.

Specialty Moves to Strike or Limit the Testimony of Black Pursuant to Daubert. (R. 49-124.) Specialty noted that Jackson retained Black as a nursing expert to testify only as to the purported nursing "standard of care." (R. 50.) Jackson's disclosure assured that "Dr. Black's testimony will be the product of reliable data, principles, and methods, all of which will be applied to the facts of this case." (R. 50.) But Specialty noted that the entirety of the testimony offered by Black was not based on sufficient data and amounted to little more than unsupported speculation and pure opinion testimony. (R. 50.) Black's opinions were prohibited ipse dixit (assertion without proof) opinions. (R. 50.) Thus, they should be stricken. (R. 50.) Additionally, Black's Third Opinion was unhelpful and irrelevant.[3] (R. 50.)

Specialty highlighted that under Daubert the burden of proof was on the proponent of the testimony, and must be established by a preponderance of the evidence. (R. 51.) Further, in adopting the federal Daubert standard, Florida intended to prohibit the admission of pure opinion testimony. (R. 51.)

Specialty noted Black's lack of substantive work experience in a Florida nursing home, and that she had never been an employee of a nursing facility. (R. 53.) Instead, she was a longtime professor teaching

[3]On appeal, Jackson challenges only the exclusion of Opinions 1-3. (IB. 25-33.)

nursing at the University of Nebraska. (R. 53.) Despite not practicing Chapter 400-governed nursing in the State of Florida, Black claimed to have become familiar with Chapter 400 through the various attorneys she worked with over the years. (R. 53.) She did not review any articles or conduct any verifiable research in coming to her opinions. (R. 53.) And she admitted that her proffered opinions were derived solely from the review of records provided to her by Jackson's counsel. (R. 53.) Black admitted not reviewing the medical examiner's records. (R. 460.)

Specialty's motion addressed Black's unsupported and ipse dixit opinions in detail with specific page/line citations. (R. 54-61.)

Specialty Argues Black's First Opinion of Failure to Monitor Was Improper Ipse Dixit. (R. 54-56.) Black opined that the Facility failed to monitor Norris after administering "short acting narcotics." (R. 54.) Specialty noted that as was common among all of her "standard of care" opinions, the First Opinion was pure conjecture and unsupported by the medical records. (R. 54-55.) Black testified as to her First Opinion:

> A. Okay. Yeah. On the night of the 20th, 21st, however we pull these together Mr. Norris was given his extended release, let me help you out, it's on Page 16 of my notes, he was given his extended release oxycodone for no pain. He has no pain. So I'm looking at Tuesday, the 20th at 21:00, the pain level is zero and he is given the extended release oxycodone per orders, I understand.
> Then the next line down says, Norco, which is hydrocodone. That one is for breakthrough pain, and it is recorded as the pain

> is at a six, and at 12:15 in the morning he was given Norco. So that's on top of his extended-release hydrocodone.
> Two things need to happen for that. Number one, you need to make sure it worked so you need to come back and make sure the patient's pain is relieved. And number two, you need to monitor for the side effects of all of that narcotic. 10 milligrams of hydrocodone is a pretty substantial dose of hydrocodone. And, of course, one of the side effects is respiratory depression. I saw nothing charted about this event at all that indicated the nurse went back at 1:15 in the morning to see what his pain relief was, to document that he was asleep, to check his O2 sat, they all carry the little O2 sat devices. None of that was done. So that's failure to safely give a narcotic and not monitor its effect or lack of effect.

(R. 55, 439-440.) Black noted Norris began on respiratory treatments, but she "couldn't really follow the record to figure out what they thought was going on." (R. 55, 440-441.) Despite testifying she didn't know what was going on with Norris' lungs, or "what the event was," she claimed that "the risk of respiratory depression to me seems much more obvious because he is coming in with a new condition, something is wrong with his lungs." (R. 55, 441.) Specialty also argued that Black imputed knowledge to the nursing staff without any factual basis establishing that the nurses knew something happened to Norris' lungs. (R. 56.) Finally, Black failed to comprehend Norris' longstanding documented chronic pain and prescribed treatment at the Facility for several years, stating she "didn't really get a good picture of where all this pain was." (R. 56, 443-444.) Specialty argued

that Black's First Opinion was pure ipse dixit guesswork that failed to articulate a verifiable Chapter 400 standard of care. (R. 56.)

Specialty Argues Black's Second Opinion as to Snoring Intervention Was Improper Ipse Dixit. (R. 56-58.) Black's second opinion alleged that the Facility failed to "follow up on his snoring at 2:00 in the morning." (R. 56.) Specialty again highlighted that Black's opinion was pure conjecture and unsupported by the medical records. (R. 57-58.) Black admitted basing this opinion solely on the EMS records, and that she did not know or read any depositions or reports of the staff that night as to "what the staff actually did and saw." (R. 57.) Black opined:

> A. No, it's the lack of action by the nurse that's the error. People that are on CPAP don't snore. Snoring is loss of your airway. Your tongue falls back. I can't say your tongue. I don't know if you snore. The reason for snoring is the tongue falls back in the airway. The nurse that heard him snoring should have known that he doesn't normally snore because he normally wears a CPAP. I think what happened in this case is too sedate from the medication he has been given to realize that he's got the CPAP wrong or that he took it off. Something happened between 12:15 and 2:00 when he quit using his CPAP that led to snoring. He shouldn't be snoring. He is on CPAP. She didn't follow up on that. She didn't go in the room.

(R. 57, 445.) Black alleged a no snoring "standard of care" opinion based purely on assumption and conjecture. (R. 58.) Black assumed Norris did not regularly snore; regularly wore a CPAP; wore a CPAP that evening; and removed it. (R. 58.) Specialty argued that Black's "best guesses" failed

to satisfy the Daubert standard. (R. 58.) Instead, she imposed a “because I said so” standard upon Chapter 400 nursing facilities. (R. 58.)

Specialty Argues Black’s Third Opinion as to Failure to Clear Airway Prior to CPR Was Improper Ipse Dixit and Irrelevant/Unhelpful. (R. 58-61.) Black’s third opinion was that the Facility failed to clear Norris’s airway prior to doing CPR:

> A. It’s not a contention, this is real. Before you start CPR you have to open the airway, and it’s my belief that she opened the airway by tipping his head backwards and doing mouth to mouth resuscitation or bag resuscitation, whatever she had, she blew the partial plate down his airway. He didn’t like it. Apparently it didn’t fit well and he only used it to eat. I don’t know why on that night when the staff put him to bed they didn’t remind him to take his partial plate out. I don’t know if they asked him, but it wasn’t out. Obviously, it was in his mouth.
> The only way to partial plate could have gotten down this airway is if somebody blew it into his airway. There is no way on Earth that he could aspirated his partial plate because he was not conscious at the time he still would have had a horrific cough response, horrific cough response. He would have gagged on it. And so only way this could have happened is if that was blown down his airway when he is dead because people that are in very deep comas, so we medicate people to put them into deep comas they still have a cough reflex when they suction. So I know that the cough reflex is one of the last reflexes to leave the body because we still see it in people with significant degrees of coma.
> He did not cough. He didn’t cough when she started CPR. So she blew the airway down his trachea. Now, they could ventilate him. You could get air past the partial plate, but they couldn’t pass the tube to maintain his airway because the partial plate was obstructing it.

(R. 59, 451-452.) Even more critically, Black's opinion was contrary to the Escambia County Medical Examiner, a qualified forensic medical doctor, who determined the cause of death. (R. 59-60, 453-457.) Black acknowledged that Norris was dead at the time CPR was performed. (R. 453, 456.)

> Q. And I know that you are mentioning now an alternate theory than what's listed on the death certificate. Are there any other theories that could be presented? For instance, the syncopal episode from days earlier and that this could have been some sort of seizure or syncopal episode?
> A. Could have been. Could have been.
> Q. In terms of his comorbidities is there anything, maybe issues with the heart or anything else that could have also caused what you are saying is not a choking death but just some other death?
> A. The only issue with his medical condition is his morbid obesity. They have a very high risk of respiratory failure, sleep apnea. He could have, when he took his CPAP off, just occluded his airway. That snoring event could have worse, and worse, and worse till he fully occluded his airway and then died. That's a possible too.
> …
> Q. So it sounds to me like you disagree with the medical examiner, what their investigation was and what their conclusion was?
> A. I disagree. Legally I can't change the cause of death but I disagree because when I saw by EMS they could ventilate him then I knew that he did not die of airway obstruction from the partial plate. If EMS would have said, we can't ventilate him with a bag, we can't force air into him then I have a different opinion. Then it was obstruction from the partial plate.
> Q. And do you recall if you received the medical examiner's records or reports and to what extent?
> A. I don't know. Hang on. I have a crime screen investigation thing. I don't know that I have the medical examiner's report. It

> doesn't ring a bell. No, but I presume it speaks to the partial plate in his airway because that's what his conclusion was.
> Q. And again that's a conclusion that you don't necessarily agree with and that you presented a different theory.
> A. Exactly.

(R. 60, 457-460.) Specialty argued that not only was this opinion again pure conjecture, it was irrelevant and unhelpful. (R. 60-61.) Black opined that "it's my belief" that "she blew partial plate down his airway." (R. 59.) But she admitted that there was nothing in the records to show that the partial plate was blown down the airway per her theory. (R. 58-61.) She acknowledged that she was providing an alternate theory as to what was listed on the death certificate and determined by the Escambia County Medical Examiner as the cause of death. (R. 60, 62, 459-460.) She admitted she never reviewed the Examiner's report. (R. 60, 460.) She testified that multiple other theories "could have" explained his death, and as a nurse she could not fill out a death certificate. (R. 457, 460.) Ultimately, she had no idea why he was wearing his partial plate that evening. (R. 60, 457.) And despite providing a CPR standard of care, as opposed to a nursing standard of care, she did not cite to any state or federal regulations, or any guideline whatsoever, governing CPR. (R. 61.)

Jackson Files Opposition to Specialty's Motion to Strike, Arguing Black's Credentials Warranted Admission of Her Opinions. (R. 125-228.) Jackson argued that Black was highly competent and qualified to offer her

opinions on the standard of care for nurses as well as causation. (R. 126-129.) Jackson claimed that Black's opinions were not wholly speculative or based on conjecture. (R. 129.) Jackson rejected that Black's opinions were based on an improper stacking of inferences and argued that professional inferences were allowed. (R. 129.)

Trial Court Invites Parties to Submit Page-Line Deposition Excerpts of Black's Testimony for Purposes of Pending Daubert Motions. (R. 357-358.) At the status conference held a month after Specialty's Daubert motion was filed, the court invited the parties to submit page-line deposition testimony supporting or opposing the motion:

> [THE COURT:] Now, the fact that the other side doesn't like that fact, that's what the jury is for; but there's a difference between speculation on nothing, as opposed to an opinion based on fact, even if it's a fact that the other side disagrees, something to that effect.
> So I ask you now, if you have the answer, that's fine. If not, I'll ask you to just email tomorrow with the statement of conflict, the page numbers and the lines. I know you cited some stuff and quoted, one testimony for a good bit already in the motion, but if there's any other page numbers or lines that are not already cited in your motions, I would like to know what to focus on. I'm going to read the whole thing – I have them all printed out.
> The same with the non-movants. If there's any page numbers that say – okay, you can read the whole thing, but if you just focus on this and this, you can [c]ite this to deny the motion. It's not based on speculation, or conjecture, the end. Let a jury decide. Something like that. I just need the page numbers. If you want to give me the lines, that's fine.
> MS. FRUSCIENTE: This is for both Daubert motions.

THE COURT: Both Daubert challenges, Daubert motions. Is it too much to ask if I can get that by tomorrow, or Wednesday. How about the close of business on Wednesday?
MR. DEAN: Tomorrow is fine, Your Honor. Thank you.

(R. 357-358.)

Specialty provided page-line citations of Black as examples of deposition testimony that failed the Daubert standard and were based on speculation and lack of evidence:

i. Examples of key opinion based speculation and lack of evidence:

1. 42:22-43:17 (the nurse is providing speculatory medical opinions, including that the decedent was experiencing some sort of lung "issue" without adequate evidence and being unable to identify the purported issue: "So adding to the story, when he came back from the hospital after that seizure event during the hospital stay they started him on respiratory treatments. I couldn't really follow the record to figure out what they thought was going on … So knowing that there is something that happened to his lungs, because he starts a couple inhalers albuterol and cortisone inhaler, and he starts his breathing treatment his lungs were not what they once were for whatever reason, whatever was going on, if it was aspiration, I don't know what the event was. . . . because he is coming in with a new condition, something is wrong with his lungs.").

2. 43:18-44:4 (the nurse is providing speculatory medical opinions and is unable to support with records: “Q. And are you able to point where in the chart the order for these two drugs is located and who wrote that order? A. No, I cannot. I can tell you that on Page 17 is where I found it in the treatment record, ordered on the 17th of December. My belief was that it came out of the hospital stay, but I couldn’t find the order per se. I’m not critical of the order at all. I’m not critical of the meds at all. I’m just stating that the nurses knew that something had happened to this guy’s lungs because he is on these two inhalers and this breathing treatment program.”).
3. 45:8-13 (the nurse is providing further medical opinion supported by speculation: “This all just fits. I’m not critical of any of the respiratory care. It’s just the background on me where I know he had been getting Norco in the past and had no problems with it, but this night he did. And the back story is all this respiratory change that happened while he was hospitalized.”).
4. 45:18-24 (the nurse is providing further medical opinion supported by speculation: “Yeah, I have a feeling that came from the hospital because I never saw much documentation from his primary provider in the nursing home. I couldn’t find many doctor records at all. So that’s my guess is he was admitted the 14th of December for what they

thought was a seizure and they started doing a fair amount of respiratory care in the hospital.").

5. 46:10-13 (the nurse is speculating on medical issues/location of pain: "So it possibly was in the non-paralyzed side but I didn't really get a good picture of where all this pain was. I saw some documentation about it quite a bit earlier in the story.").
6. 46:24-47:5 (the nurse is speculating and does not have all facts. "Right. I haven't seen the deposition of any of the staff that night and I did not have any kind of narrative notes or incident report to read about what the staff actually did and saw.").
7. 47:15-22 (the nurse is speculating that the decedent does or does not snore if not wearing a CPAP, or always wears the CPAP, without adequate facts: "The nurse that heard him snoring should have known that he doesn't normally snore because he normally wears a CPAP. I think what happened in this case is too sedate from the medication he has been given to realize that he's got the CPAP wrong or that he took it off. Something happened between 12:15 and 2:00 when he quit using his CPAP that led to snoring."). a. 48:23-49:2 (the nurse later admits she does not know how often decedent wore the CPAP).
8. 53:9-22 (the nurse is speculating and then admitting does not know what happened: "I don't know why on that night when the staff put him

to bed they didn't remind him to take his partial plate out. I don't know if they asked him, but it wasn't out.").

9. 54:19-22 (the nurse is speculating that an unknown person must have done something without support in the records).
10. 55:17-56:7 (the nurse is speculating an entire series of events without any support in the records, admitting "I'm making presumptions here, I realize").
11. 63:2-5 (the nurse provides a final irrelevant opinion that she "cannot link directly to damages").

ii. Examples of relying upon unreliable testimony of the decedent's mother (who was not with decedent on the night of his passing):

1. 56:19-22 (the nurse is relying upon deposition testimony of decedent's mother).

iii. Examples of not knowing or misunderstanding the facts:

1. 62:2-12 (the nurse admits does not have medical examiner records in file).

(R. 255-258.)

Despite the court's invitation, Jackson did not submit page-line citations of Black's deposition to the court for consideration in evaluating the parties' Daubert motions.

Jackson Confirmed No Medical Causation Testimony Would be Elicited from Black at Trial.

> THE COURT: And then verbally what you told me is the nurse, the Ph.D., the doctor, or whatever, you know, but if she wants to give an opinion on the cause of, you know, snoring. I suspect Plaintiff is not going to spend their time trying to prove what causes snoring, I think they're going to talk about the standard of care and whether or not it was breached or not. So, you know, your framing of the issues may not be how the issues that they actually present – so I have to go back – I have to mesh all that together.
> MS. FRUSCIENTE: Yes, sir.
> THE COURT: Really for purposes of the motions you filed, it's a question of, you know, had the proponent of the expert testimony, you know, put on enough to be able to survive the challenge. That's really what it comes down to, so it's just going to take a heavy, heavy read of the depos to get that determined, but I hear everything you say – but, of course, then you get into the standard for DV, or a JNOV is different from Daubert. I hear it loud and clear. That doesn't mean – well, we can deal with it on the back end, so why does it matter now? It matters enough to file it, that's why you have a Daubert challenge now.
> You're supposed to be the guard keeper, right? The gate keeper? To not let this stuff go to the jury in the first place, but it's also not to be – well, we're just – the jury is not good enough to figure this out on your own, so I get it – and you can have that done by the end of this week.
> MR. DEAN: So that ends – prohibited from introducing duplicative testimony. The nurses are going to give nursing standard of care opinions, the doctor is going to give causation opinions on my direct. What goes on at depositions, is depositions, but I understand what I can do and what I can't do. So if you're reading something in a vacuum, you might go, well, he's not asking these questions, I get it, but, you know, if there's any confusion, let me just say it here, I'm not eliciting medical causation opinions from my nurse. I'm not eliciting causation opinions about snoring. You know, everything is the standard of care, and how the nurse violated it, and how those violations was the cause of death. That's my focus at trial.

(R. 362-364.) (emphasis added). The trial court clarified, "[t]he nurse is for the nursing standard of care, she's a professor, but she's no doctor for causation of death." (R. 365.)

Trial Court Grants Specialty's Daubert Motion. (R. 236-239.) The trial court's order stated in pertinent part as follows:

> In this matter, Defendant seeks to preclude Joyce Black, Ph.D. from providing an opinion that the facility, with regard to Mr. Norris, failed to (i) safely give him a narcotic and not monitor any effect (ii) fail to investigate the reason for his snoring at 2:00 AM, (iii) clear the airway before CPR was performed, and (iv) failing to advise his physician about the level of blood sugar. (Depo. J. Black pp. 42, 47, 51, 52-53, 64). Notably, the opinion that Mr. Norris' physician should have been notified about lab values is not directly related to the cause of death. Moreover, the opinion the nursing staff failed to clear the airway before performing CPR concedes that Mr. Norris had already died. Otherwise, the opinion of Joyce Black Ph.D that the Defendant failed to the effect of the narcotic is based upon review of records that does not show annotations by the nursing staff following the administration. To be sure, the witness seemingly concludes the absence of any documentation excludes all possibilities other than a lack of attention or care. But the Plaintiff is not prohibited from relying on inferences to form an opinion. This is not a situation where the expert relies on something that is wholly unsupported by the record; an expert can rely on selected portions of contradicting evidence to support an opinion. And although there is some conjecture about what others supposedly knew and when they knew it, there are no submitted reliable principles and methods upon which the standard of care is defined to identify when and the frequency of any monitoring.
>
> Similarly, the witness finds fault with the Defendant and failing to investigate the snoring incident at approximately 2:00 AM. While there is much conjecture and speculation about what the nursing staff would have found had they done so, the opinion to check on the patient who snores lacks any reliable principles

> and methods upon which the standard of care operates. Even assuming her conclusion about what might have happened had an investigation been done, there's nothing to substantiate the expert's opinion to establish that the standard of care requires an investigation based on snoring.
> The opinions of Joyce Black PhD that the Defendant's nursing staff failed to clear the airway before CPR was performed to advise his physician about the level of blood sugar will not assist the trier of fact in determining a fact in issue. The opinions that the Defendant failed to monitor any effect of the narcotic and to investigate the reason for his snoring at 2:00 AM is not a product upon reliable principles and methods. Giaimo v. Florida Autosport, Inc., 154 So. 3d 385 (Fla. 1st DCA 2015). Therefore, the motion is GRANTED to preclude Joyce Black, Ph.D. from providing her offered opinions as identified in her deposition.
>
> (R. 237-238.)

Jackson Does Not Move for Rehearing but Instead Files Emergency Motion for Continuance of Trial to Obtain New Nursing Expert. (R. 240-241.) Specialty opposed Jackson's motion on several grounds including substantial prejudice and failure to meet the standard for a trial continuance. (R. 243-245.) The trial court denied the motion for continuance. (R. 247.)

Specialty Files Unopposed Emergency Motion for Summary Judgment. (R. 315-324.) Specialty noted that Jackson alleged that it failed to develop an adequate care plan and properly monitor and supervise the care and treatment of Norris, which allegedly contributed to Norris choking on his dentures and passing away. (R. 316.) In order for Jackson to prevail

at trial, Jackson had the burden of showing that Specialty breached a duty of care to the resident. (R. 317.) Under Florida law, Jackson had to prove at trial that Specialty fell below the applicable nursing home standard of care through a nursing standard-of-care expert. (R. 317.) Given Jackson's acknowledgment that this burden could not be met, summary judgment in favor of Specialty was warranted. (R. 318.)

Specialty filed the Declaration of Dr. Inna Sheyner, MD, CMD, AGSF supporting its motion for summary judgment. (R. 328-333.) Dr. Sheyner averred she was licensed to practice medicine in the State of Florida, and for the past 29 years had focused on internal and geriatric medicine. (R. 331.) Dr. Sheyner listed all the medical records reviewed, including Ascension Sacred Heart Pensacola; HCA Florida West Hospital; Mizell Memorial Hospital; pharmacy records; and dental records. (R. 332.) Dr. Sheyner also reviewed Escambia County EMS records; Escambia County Sheriff's Office reports and records; the death certificate of Norris and District One Medical Examiner's records. (R. 332.) Dr. Sheyner also reviewed all the deposition transcripts in the case. (R. 332.) Dr. Sheyner opined based on her review of the above materials, and her education, training and experience in a nursing home setting. (R. 332.) She opined that at no time did Specialty or anyone acting on its behalf breach the

applicable standard of care under Chapter 400, Florida Statutes, governing nursing homes in Florida. (R. 332.)

Trial Court Grants Final Summary Judgment. (R. 248-250.) The trial court noted that it granted Specialty's motion to strike or limit the testimony of Black, finding that "Nurse Black's opinions that Defendant failed to monitor any effect of the narcotic and to investigate the reason for snoring at 2:00 AM are not a product upon reliable principles and methods." (R. 249.) The trial court noted that Jackson's motion to continue the trial was "both deficient as to form as well as substance, with no indication that the parties were not afforded a reasonable time to be prepared for trial." (R. 249.) Further, that Jackson agreed that "without a standard of care expert, Plaintiff would be unable to prove a required element of the case, and a directed verdict would be granted in favor of the Defendant at trial." (R. 249.) The trial court granted final summary judgment based upon Jackson's inability to prove a material fact. (R. 250.)

**SUMMARY OF THE ARGUMENT**

Jackson's primary argument to the trial court was that Black's qualifications rendered her opinions admissible. Jackson never identified the methodology used by Black, much less explained to the trial court why this methodology was allegedly reliable. On appeal, Jackson now contends that Black employed a "differential diagnosis" methodology, and this

methodology rendered Black's opinions reliable and admissible. As this argument is made for the first time on appeal, it should not be considered by this Court.

On the merits, Jackson fails to show that the trial court abused its discretion in excluding Black's Opinions One-Three. Black's testimony repeatedly highlighted that she was speculating, guessing, or hypothesizing as to the basis of her opinions. Her First Opinion – that the nursing staff failed to monitor Norris as to the effects of narcotics – was pure ipse dixit guesswork. It failed to articulate a verifiable Chapter 400 standard of care. It was based on assumptions made from not seeing entries on Norris' chart. The trial court did not abuse its discretion in excluding the First Opinion.

Black's Second Opinion – that the nursing staff failed to follow up on snoring – was also another "because I said so" opinion. Black had no information as to whether Norris regularly snored, and admitted he was free to remove his CPAP. Her opinion was based on a series of assumptions and best guesses evidencing the lack of a factual foundation. The trial court did not abuse its discretion in excluding Opinion Two.

Black's Third Opinion as to the failure to clear the airway was also properly stricken. Black opined as to the cause of Norris' death which was contrary to the cause of death determined by the Medical Examiner. She admitted never reviewing the Medical Examiner's records, and that she

was not qualified as a nurse to determine the cause of death on a death certificate. Despite Jackson's assurance to the trial court that her nursing expert would not opine as to medical causation, that is precisely the nature of Opinion Three. And Norris was admittedly already dead by the time the alleged failure to clear his airway occurred. So the testimony would admittedly not assist the jury in determining a fact at issue. Black testified that whether the nurse cleared his airway would impact only how his cause of death would be listed on the death certificate – a determination she admitted she was not qualified to make. Black's Third Opinion was properly excluded by the trial court.

**ARGUMENT**

**STANDARD OF REVIEW.** This Court reviews a trial court's decision to admit or exclude evidence under Daubert for an abuse of discretion. *Huggins v. Siegel*, 336 So. 3d 58, 60 (Fla. 1st DCA 2021).

Although summary judgment is ordinarily reviewed de novo, the nature of Jackson's challenge eliminates any independent significance of that standard here. Jackson's sole basis for challenging summary judgment is the exclusion of Black's testimony. No independent error in the summary judgment analysis is raised itself. Jackson conceded below that without a nursing standard-of-care expert, she could not meet her burden of proof on her Chapter 400 claim against Specialty. (R. 249–250.) Accordingly, the

summary judgment stands or falls with the Daubert order. If this Court affirms the exclusion of Black's opinions, the summary judgment should be affirmed.

It is the appellant's sole burden to present an adequate record demonstrating reversible error to the appellate court. *Schroeder v. MTGLQ Invs., L.P.*, 290 So. 3d 93, 95 (Fla. 4th DCA 2020). Further, arguments presented for the first time on appeal are waived. *Adkison v. Morey*, 239 So. 3d 205, 207 (Fla. 1st DCA 2018) ("Because the argument made on appeal was never presented to the trial court, it is waived."). To the extent Jackson raises arguments on appeal that were never presented to the trial court, those arguments are unpreserved and are not properly before this Court. *Aills v. Boemi*, 29 So. 3d 1105, 1108–09 (Fla. 2010). Additionally, arguments may not be raised for the first time in a reply brief on appeal. *Carrasquillo v. Holiday Carpet Serv.*, 615 So. 2d 862, 863 (Fla. 3d DCA 1993). Thus, the failure to raise an issue and present argument in the initial brief constitutes waiver and abandonment of the issue.

In this regard, Jackson does not raise the denial of her emergency motion for a continuance of trial as an issue on appeal. Ordinarily, a trial court's denial of a trial continuance would be reviewed for an abuse of discretion. *Krock v. Rozinsky*, 78 So. 3d 38, 41 (Fla. 4th DCA 2012). But

here, Jackson has waived and abandoned any consideration of this issue on appeal.

Finally, this Court may affirm on any basis supported by the record, even if the trial court did not rely on that basis, under the tipsy coachman doctrine. *Dade Cnty. Sch. Bd. v. Radio Station WQBA*, 731 So. 2d 638, 644–45 (Fla. 1999).

## *I. JACKSON'S ARGUMENT AS TO METHODOLOGY EMPLOYED BY BLACK IS NOT PRESERVED*

Florida law is well settled that to preserve an issue for appellate review, the specific legal argument must be presented to the lower court. *Aills v. Boemi*, 29 So. 3d 1105, 1108–09 (Fla. 2010). A party may not make one argument to the trial court, lose, and then present a different argument on appeal, regardless of how the new argument might relate to the same general subject matter. *Tillman v. State*, 471 So. 2d 32, 35 (Fla. 1985).

### A. Jackson's Differential-Diagnosis Argument Was Not Made Below to Support Admission of Opinions One-Three

In the trial court, Jackson argued that Black's qualifications and credentials warranted admission of her opinions. And that Black was permitted to opine based on her review of the medical records. Jackson did not address Black's methodology, much less how she arrived at each opinion. Now, on appeal, Jackson argues for the first time that Black employed a "differential diagnosis" methodology in arriving at her causation

opinions. (IB. 24.) As the trial court was never presented with any arguments as to the methodology allegedly employed by Black, let alone that the methodology rendered her opinions reliable and admissible, Jackson cannot now raise these arguments for the first time on appeal to show that the trial court abused its discretion. Instead of moving for a continuance to obtain a new expert, Jackson could have simultaneously or alternatively moved for rehearing setting forth arguments as to why Black's "differential diagnosis" methodology was reliable and warranted admission of her opinions. Jackson's failure to raise any arguments as to the methodology allegedly relied on by Black, either in opposition to the Daubert motion or on rehearing, precludes consideration of the argument for the first time on appeal. *Pensacola Beach Pier, Inc. v. King*, 66 So. 3d 321, 324 (Fla. 1st DCA 2011) (holding that an appellant failed to preserve an issue for review when the trial court's error appeared for the first time on the face of the order on appeal and appellant chose not to file a motion for rehearing).[4]

[4] Even if considered, the differential diagnosis method is a causation technique (ruling in possible causes and ruling out the alternatives) that cannot supply a missing nursing standard of care. See, e.g., *Chapman v. P&G Distrib., LLC*, 766 F.3d 1296, 1308-11 (11th Cir. 2014). As noted infra, Black was to provide her nursing standard-of-care opinion, not causation opinions. Jackson cannot now attempt to justify Black's excluded nursing opinions with after-the-fact causation methodologies.

## *II. THE TRIAL COURT DID NOT ABUSE ITS DISCRETION IN EXCLUDING BLACK'S NURSING STANDARD OF CARE OPINIONS*

As set forth herein, this Court should not consider Jackson's argument, raised for the first time on appeal, as to the "differential diagnosis" methodology employed by Black. And on the merits, Jackson fails to show that the trial court abused its discretion in excluding Black's First, Second and Third Opinions under Daubert.

### A. The Trial Court Applied the Correct Legal Standard Under Section 90.702 and Daubert

Section 90.702, Florida Statutes, establishes the three-part framework governing the admissibility of expert testimony. Under that provision, a witness qualified as an expert may testify in the form of an opinion if: (1) the testimony is based upon sufficient facts or data; (2) the testimony is the product of reliable principles and methods; and (3) the witness has applied the principles and methods reliably to the facts of the case. § 90.702(1)-(3), Fla. Stat.; *Daubert v. Merrell Dow Pharms., Inc.*, 509 U.S. 579 (1993). The burden to establish admissibility of the expert's opinion falls squarely on the proponent of the evidence. *McCorvey v. Baxter Healthcare Corp.*, 298 F.3d 1253, 1257 (11th Cir. 2002).

The trial court has broad discretion in exercising its role as gatekeeper and evaluating an expert's opinion for admissibility. *Booker v. Sumter Cty. Sheriff's Office/N. Am. Risk Servs.*, 166 So. 3d 189, 192 (Fla.

1st DCA 2015). Florida courts have emphasized the proper analysis to be applied under Daubert:

> Under Daubert, a trial judge has a gatekeeping role to "ensure that any and all scientific testimony or evidence admitted is not only relevant, but reliable." 509 U.S. at 589. The trial judge is "charged with this gatekeeping function 'to ensure that speculative, unreliable expert testimony does not reach the jury' under the mantle of reliability that accompanies the appellation 'expert testimony.'" *Rink v. Cheminova, Inc.*, 400 F.3d 1286, 1291 (11th Cir. 2005) (citation omitted).
>
> A trial judge must make "a preliminary assessment of whether the reasoning or methodology underlying the testimony is scientifically valid and of whether that reasoning or methodology properly can be applied to the facts in issue." Daubert, 509 U.S. at 592-93. This basic gatekeeping obligation applies not only to scientific testimony, but "to all expert testimony." *Kumho Tire Co., Ltd. v. Carmichael*, 526 U.S. 137, 147, 119 S. Ct. 1167, 143 L. Ed. 2d 238 (1999).
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> The Supreme Court in Daubert outlined a list of factors that bear on the reliability inquiry: (1) whether the theory can be or has been tested; (2) whether the theory or technique has been subjected to peer review and publication; (3) the known or potential rate of error of a particular scientific technique, as well as the existence of standards controlling the technique's operation; and (4) general acceptance in the scientific community. 509 U.S. at 593-94. The Daubert "test of reliability is flexible, and Daubert's list of specific factors neither necessarily nor exclusively applies to all experts or in every case." Kumho Tire, 526 U.S. at 141 (internal quotation marks omitted).
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> "[T]he test under Daubert is not the correctness of the expert's conclusions but the soundness of his methodology." Daubert v. Merrell Dow Pharm., Inc., 43 F.3d 1311, 1318 (9th Cir. 1995) ("Daubert II"). However, an expert's opinion must be based upon "knowledge," not merely "subjective belief or unsupported speculation." Daubert, 509 U.S. at 590. Nothing in Daubert requires a court "to admit opinion evidence that is connected to existing data only by the ipse dixit of the expert," and "[a] court may conclude that there is simply too great an analytical gap

between the data and the opinion proffered." GE v. Joiner, 522 U.S. 136, 146, 118 S. Ct. 512, 139 L. Ed. 2d 508 (1997).

*Kemp v. State*, 280 So. 3d 81, 88 (Fla. 4th DCA 2019).

Florida courts applying Daubert have routinely found an expert's testimony inadmissible if it is grounded on speculation, conjecture, or incorrect assumptions. *Doctors Co. v. Dep't of Ins.*, 940 So. 2d 466, 470 (Fla. 1st DCA 2006). In *Siegel v. Cross Senior Care, Inc.*, 239 So. 3d 738, 743 (Fla. 3d DCA 2018), the expert created and drew inferences solely from the medical records which were unsupported or flatly contradicted.[5] The expert's entire theory was premised on an opinion that the decedent died of pneumonia. *Id.* However, this was contradicted by the cause of death given in the death certificate. The appellate court affirmed the trial court's finding that "the opinion of this doctor is pure ipse dixit." *Id.*

**B. Trial Court Correctly Evaluated Black's Opinions as to Reliability and Methodology**

Here, the trial court evaluated whether Black identified reliable principles and methods underlying her opinions. (R. 236-239.) The trial court identified the specific deficiency in Black's testimony, namely the absence of any articulable methodology or external standard supporting

[5] The expert excluded in *Siegel* was Dr. Lee A. Fischer, Jackson's "causation" expert in this matter. (R. 69.)

her opinions, and excluded the testimony on that basis. There is no colorable argument that the trial court applied the wrong legal standard.

Jackson urges this Court to treat the inquiry as one of "competency rather than methodology." (IB. 13-20.) In other words, there is no reason to examine Black's methodology because of her experience. That formulation, if adopted, would collapse the qualification prong into the reliability prong, rendering Florida's statutory three-part structure a nullity. § 90.702(1)-(3), Fla. Stat. A rule that permits any qualified expert to offer any opinion within the general scope of her field, without independently demonstrating that the opinion rests on reliable principles and methods, would read the second and third requirements out of the statute entirely.

Florida courts applying Daubert have not permitted such an approach. In *Giaimo v. Florida Autosport, Inc.*, 154 So. 3d 385 (Fla. 1st DCA 2014), this Court reversed the admission of an expert's opinions when the expert reviewed the medical records and testified his opinions were based on his review. This Court found this was insufficient under Daubert:

> In 2013, the Florida Legislature modified section 90.702 "to adopt the standards for expert testimony in the courts of this state as provided in Daubert v. Merrell Dow Pharmaceuticals, Inc., 509 U.S. 579, 113 S. Ct. 2786, 125 L. Ed. 2d 469 (1993), General Electric Co. v. Joiner, 522 U.S. 136, 118 S. Ct. 512, 139 L. Ed. 2d 508 (1997), and Kumho Tire Co., Ltd. v. Carmichael, 526 U.S. 137, 119 S. Ct. 1167, 143 L. Ed. 2d 238 (1999), and to no longer apply the standard in Frye v. United

States, 293 F. 1013 (D.C. Cir. 1923). . . . As amended, section 90.702 now provides:

If scientific, technical, or other specialized knowledge will assist the trier of fact in understanding the evidence or in determining a fact in issue, a witness qualified as an expert by knowledge, skill, experience, training or education may testify about it in the form of an opinion or otherwise, if:

(1) The testimony is based upon sufficient facts or data;

(2) The testimony is the product of reliable principles and methods; and

(3) The witness has applied the principles and methods reliably to the facts of the case.

§ 90.702, Fla. Stat. The Legislature's adoption of the Daubert standard reflected its intent to prohibit "pure opinion testimony, as provided in Marsh v. Valyou, 977 So. 2d 543 (Fla. 2007)[.]" Ch. 13-107, § 1, Laws of Fla; see Charles W. Ehrhardt, 1 Fla. Prac., Evidence § 702.3 (2014 ed) ("In adopting the amendment to section 90.702, the legislature specifically stated its intent that the Daubert standard was applicable to all expert testimony, including that in the form of pure opinion.") (footnote omitted).

As to subsection (1) of the statute, Dr. Lee based his opinion on the medical records including diagnostic studies from the doctor who treated Giaimo for his 2009 non-work related motor vehicle accident. Dr. Lee arguably had sufficient facts and data upon which he could base his opinion. We need not make a conclusive decision on this requirement, however, because we find that the second and third requirements were not met.

As to subsections (2) and (3), we review them in tandem, finding a lack of "reliable principles and methods" and thereby no reliable application of them. When Dr. Lee was asked how he arrived at the percentages attributable to Giaimo's pre-existing condition and those attributes to the workplace injury, he explained that "when I was asked and thought about it, that is the answer that I came up with." This testimony provides no insight into what principles or methods were used to reach his opinion, and Dr. Lee did not demonstrate that he applied any such principles or methods to the facts of this case.

In rejecting Giaimo's argument that Dr. Lee's testimony was pure opinion testimony, the JCC explained that Dr. Lee's

> "opinion was based on his experience and treatment of the claimant and thorough review of the medical and treatment records of Dr. Rodrigo Aubunag, M.D., who treated the claimant's injuries sustained in the 2009 non-work related care accident." This basis for Dr. Lee's opinion, however, is precisely what makes it pure opinion testimony under Marsh, 977 So. 2d at 548-49. Testimony of this type, though previously acceptable as pure opinion under Marsh, no longer suffices under section 90.702. The 2013 Legislature has made clear that the admissibility of expert opinions requires that the requirements of Daubert be met. Because no basis in the record exists to support that Dr. Lee's testimony was "the product of reliable principles and methods" and that Dr. Lee applied such principles, the JCC's conclusion that Dr. Lee's testimony met Florida's newly-adopted Daubert standard is erroneous.

Id. at 388-89; see also *Philip Morris USA, Inc. v. Naugle*, 337 So. 3d 13, 18 (Fla. 4th DCA 2022) ("[T]he law requires more than experience alone; it requires the court to assess whether the expert's 'reasoning or methodology properly can be applied to the facts in issue.'").

Jackson's reliance on Judge Garza's concurrence in *Adams v. Laboratory Corp. of America*, 760 F.3d 1322, 1338 (11th Cir. 2014) to support her "competency rather than methodology" argument is misplaced. *Adams* involved a negligence action against laboratory cytotechnologists for failing to identify abnormalities in a plaintiff's Pap smears, leading to an alleged delay in a cancer diagnosis. *Id.* at 1325. In *Adams*, the plaintiff's expert, Dr. Rosenthal, traveled to the laboratory in question and reviewed the Pap smear slides under a microscope, before opining that the standard of care was not met and that the abnormal cells should have been

identified by the laboratory cytotechnologists. *Id.* at 1326. The district court (N.D. Ga.) took issue with the evaluation and excluded the opinion. *Id.* at 1326-27. The district court ruled, inter alia, that a blinded review of the slides was the standard set by the profession. *Id.* The federal appellate court disagreed with each of the district court's four grounds for exclusion, determining that the expert was qualified, the expert used a methodology that was reliable [the non-blinded Pap smear review], and the expert's testimony would assist the trier of fact. *Id.* at 1331-36.

Rather than relying upon the *Adams* majority opinion, Jackson relies heavily upon Judge Garza's concurrence. (IB. 14-17.) Judge Garza disagreed with the majority's reasoning only as to the discussion on methodology. *Id.* at 1336 (Garza, J., concurring). Judge Garza opined that "Dr. Rosenthal's testimony on the standard of care and its alleged breach in this case is admissible because her qualifications and experience are sufficient." *Id.* In his view, both the cytotechnologists' screening task and Dr. Rosenthal's review "involve only the application of professional judgment, not 'methodology.'" *Id.* at 1337. He drew support from *McDowell v. Brown*, 392 F.3d 1283 (11th Cir. 2004) (applying Georgia law), decisions from other circuits, and the advisory committee's note to Rule 7026, inter alia. Thus, Judge Garza's concurrence argues for a Daubert analysis without any

methodology scrutiny, in direct contradiction of Section 90.702, Florida Statutes.[6]

Jackson's reliance on *Baan v. Columbia County*, 180 So. 3d 1127 (Fla. 1st DCA 2015) is similarly misplaced. In *Baan*, EMS answered a 911 call about an 11-month-old in respiratory distress. *Baan*, 180 So. 3d at 1129. EMS personnel left within ten minutes after showing the child's aunt how to use a nebulizer, and about 50 minutes later, a second 911 call reported that the child was not breathing. *Id.* The child died the next day. *Id.* The evidence conflicted on the first EMS visit. A neighbor testified that the EMS personnel never examined or touched the child, while the EMS report recorded normal vital signs. *Id.* The plaintiff's expert, Dr. Tulsiak, opined that EMS breached the standard of care, and violated its own "Respiratory Distress" protocol, by failing to transport the child to a hospital on the first run. *Id.* at 1130. "The trial court concluded that Dr. Tulsiak had rejected evidence he should have accepted as true (the EMS report) and that 'the only evidence as to the child's true respiratory status was recorded by the

[6]Judge Garza quotes the advisory committee's note, which states that "[i]f the witness is relying solely or primarily on experience, then the witness must explain how that experience leads to the conclusion reached, why that experience is a sufficient basis for the opinion, and how that experience is reliably applied to the facts." *Adams*, 760 F.3d at 1340 (quoting Fed. R. Evid. 702 advisory committee's note (2000 Amendments)). Black did none of these things.

paramedics,' so that Dr. Tulsiak's opinions were 'premised on speculation based on an ultimate injury and manufactured facts.'" *Id.* at 1131.

Initially, *Baan* does not support Judge Garza's opinion as stated in his *Adams* concurrence – that expert methodology need not be examined if an expert is competent and qualified. To the contrary, the *Baan* opinion highlights that the Florida Legislature has adopted the Daubert standard for admissibility of an expert's testimony, as codified in the amended Section 90.702, Florida Statutes. *Baan*, 180 So. 3d at 1132-33. The *Baan* opinion explained:

> Under Daubert, although "an expert may be qualified by experience," it does not follow "that experience, standing alone, is a sufficient foundation rendering reliable any conceivable opinion the expert may express." United States v. Frazier, 387 F.3d 1244, 1261 (11th Cir. 2004) ("If admissibility could be established merely by the ipse dixit of an admittedly qualified expert, the reliability prong would be, for all practical purposes, subsumed by the qualification prong."); see also Charles W. Ehrhardt, 1 Fla. Prac., Florida Evidence § 702.3 (2015 ed.) ("When an expert is relying primarily on experience, the witness must explain how that experience leads to the opinion, why the experience is a sufficient basis for the opinion and how that experience is reliably applied to the facts." (citing Am. Gen. Life Ins. Co. v. Schoenthal Family, LLC, 555 F.3d 1331 (11th Cir. 2009), and Primiano v. Cook, 598 F.3d 558 (9th Cir. 2010))). Under section 90.702, as amended, while the expert's qualifications may well remain germane, an expert witness must explain the logic and relevance of the expert opinion.
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> *Baan*, 180 So. 3d at 1133.

The *Baan* court concluded that Dr. Tulsiak's opinions "amounted to much more than ipse dixit." *Id.* The court determined that Dr. Tulsiak based his opinions on medical, autopsy, and EMS records, eyewitness statements, decades of emergency medicine experience, and EMS's own transport protocol, and they were not post hoc speculation. *Id.* at 1133-34. Notably, the expert's standard-of-care opinion was admissible because, in addition to his experience, he invoked "as one salient 'reliable principle[]' EMS's own protocol requiring transport to a hospital in the event an infant was experiencing respiratory distress." *Id.* at 1133 (alteration in original). Thus, this Court found that "the record makes clear that Dr. Tulsiak's testimony was 'the product of reliable principles and methods,' and that those principles and methods were applied 'reliably to the facts of the case.'" *Id.* at 1134 (quoting § 90.702, Fla. Stat.).

Here, contrary to the facts in *Baan*, the record shows that Black's opinions were not "based upon sufficient facts or data," § 90.702(1), Fla. Stat. Rather, these opinions amounted to little more than "because I said so." Additionally, the trial court did not reject Black's opinions because she relied on records or drew inferences; rather, it expressly acknowledged that she could do both. (R. 237-238.) The trial court's ruling instead rested on the reliability prong. (R. 238.) Jackson's extended argument that experts may rely on medical records (IB. 21-24) is misplaced.

In sum, the trial court did not abuse its discretion in excluding Black's opinions that failed to meet the reliability prong.

**C. The Trial Court Properly Excluded Black's First Opinion as to Failure to Monitor**

Black opined that after the nursing staff administered pain medication, they should have monitored for side effects. She testified: "Two things need to happen . . . . Number one, you need to make sure it worked . . . . And number two, you need to monitor for the side effects of all of that narcotic. . . . None of that was done." (R. 83–84, 440.) That testimony was the sum and substance of her methodology: a review of the chart, the absence of documentation, and a conclusion that the standard of care was breached.

The trial court found Black's opinion "is based upon review of records that does not show annotations by the nursing staff following the administration" and that "the witness seemingly concludes the absence of any documentation excludes all possibilities other than a lack of attention or care." (R. 237.) The court further found that "there are no submitted reliable principles and methods upon which the standard of care is defined to identify when and the frequency of any monitoring." (R. 237–238.)

Black's testimony was quintessential ipse dixit—an opinion offered on no authority beyond the expert's own assertion. In her deposition, she

identified: (a) no specific frequency or timing for post-administration monitoring of narcotic medications; (b) no physician order requiring specific monitoring intervals; (c) no facility policy or care plan governing narcotic monitoring; (d) no nursing regulation, professional guideline, or clinical protocol defining the monitoring standard she claimed was breached; (e) no published literature supporting a particular monitoring regimen following the administration of these medications in a setting such as this; and (f) no testimony from any treating provider regarding the monitoring that was expected or customary for Norris. (R. 439-444.)

The deficiencies in Black's foundation went still further. She admitted that she "couldn't really follow the record to figure out what they thought was going on" about Norris' respiratory treatment. (R. 440-441.) She could not identify who wrote the relevant orders. (R. 441.) And she conceded that she "didn't really get a good picture of where all this pain was." (R. 444.)

In General Electric Co. v. Joiner, 522 U.S. 136, 146 (1997), the Court held that "nothing in either Daubert or the Federal Rules of Evidence requires a district court to admit opinion evidence that is connected to existing data only by the ipse dixit of the expert." *Id.* Again, Florida courts have faithfully applied this principle. See *Perez v. Bell South Telecomms., Inc.*, 138 So. 3d 492, 497–98 (Fla. 3d DCA 2014) (stating that the legislature intended to prohibit pure opinion testimony lacking a reliable

methodological basis); *Kemp v. State*, 280 So. 3d 81, 88–89 (Fla. 4th DCA 2019).

What Black offered was precisely what Joiner and its progeny prohibit: an analytical gap between the data and the opinion, bridged by nothing more than the expert's credentials and say-so. She possessed credentials as a nurse. She reviewed the chart. She noted the absence of documentation. And she leapt to the conclusion that the standard of care was breached without identifying what standard governed, where it was memorialized, how it applied to Norris, or what specific monitoring should have occurred and at what intervals. The "analytical gap" between Black's credentials and her opinions was exactly the kind of gap that Daubert's gatekeeping function is designed to identify and exclude. The trial court properly performed that function.

**D. The Trial Court Properly Found No Reliable Standard Supported Snoring Opinion**

Black's Second Opinion concerned the nursing staff's response to Norris's snoring. She opined that the nursing staff should have investigated when Norris snored because, in her view, "people that are on CPAP don't snore" and she believed he was "too sedate." (R. 445.) From this, she concluded that the standard of care required the nursing staff to intervene upon hearing Norris snore. (R. 445-450.)

The evidentiary foundation for this opinion was remarkably thin. Black acknowledged that her evidence "comes from the EMS records" only. (R. 444-445.) She candidly admitted: "I haven't seen the deposition of any of the staff that night and I did not have any kind of narrative notes or incident report to read about what the staff actually did and saw." (R. 445.) She thus rendered an opinion about what the nursing staff should have done without reviewing any evidence of what the nursing staff actually did, observed, or knew at the relevant time.

The gaps in Black's knowledge went to the heart of her opinion. She did not know how often Norris wore his CPAP or whether the device was broken. (R. 446-447.) She acknowledged that Norris had the right to refuse the CPAP. (R. 446.) She stated, "I can hypothesize but I don't know why he took the CPAP off." (R. 448.) She readily acknowledged: "I'm making a presumptions here, I realize." (R. 454.) An opinion built on hypothesis and self-acknowledged presumption is not an opinion grounded in reliable principles and methods.

Most critically, when pressed on whether the hourly checks performed by the nursing staff satisfied the standard of care, Black answered: "Yes, except for a patient who shouldn't be snoring then you need to intervene." (R. 458.) Thus, Black conceded that conducting hourly checks was within the standard of care. She then proposed an exception to that standard,

triggered by the observation of snoring in a CPAP patient, that would require additional intervention beyond the hourly monitoring. For that exception, she identified no source, no standard, no protocol, no clinical guideline, and no published literature.

Not surprisingly, the trial court found that the opinion to "check on the patient who snores lacks any reliable principles and methods upon which the standard of care operates" and that "there's nothing to substantiate the expert's opinion to establish that the standard of care requires an investigation based on snoring." (R. 238.) Again, Black failed to identify a nursing standard, regulation, protocol, textbook, peer-reviewed article, or professional guideline establishing that the observation of snoring in a CPAP patient triggered a duty of investigation or intervention beyond routine monitoring.

The analytical leap required to accept Black's opinion illustrates precisely why Daubert gatekeeping exists. The trial court was not required to accept Black's ipse dixit that snoring in a CPAP patient triggers a duty of investigation. Under Daubert and Section 90.702, the court was required to serve as a gatekeeper to ensure that expert testimony admitted before the jury rests on a reliable methodological foundation rather than simply because the expert says so.

**E. The Trial Court Properly Excluded Black's Third Opinion as to Alleged Failure to Clear Airway as Improper Ipse Dixit and Irrelevant/ Unhelpful**

Black's Third Opinion is perhaps the most problematic of all. (R. 58-61.) Despite assuring the trial court that no medical causation testimony would be offered by Black at trial, Black's Third Opinion is that Norris died from respiratory distress, not swallowing his dentures. (R. 364, 456.) Black admits that her opinion as to the cause of death is contrary to that of the Medical Examiner and, as a nurse, she cannot issue a death certificate. (R. 453-460.) Even more troubling, Black admits that Norris was already dead at the time that the Facility staff performed CPR and allegedly failed to clear his airway. (R. 456.) And she admits that clearing Norris's airway would not have made any difference. He still would have died, "but it just wouldn't have been listed on the death certificate as the cause." (R. 453.) A listing that Black admits she is not qualified to make. (R. 453.)

The trial court found that Black's opinion as to the failure to clear the airway would not assist the trier of fact in determining a fact in issue given Norris was already dead at the time. (R. 238.) There is no abuse of discretion in this decision.

On appeal, Jackson argues that "[t]his opinion established that the cause of Mr. Norris' death was a result of the breaches of the standard of

care Dr. Black identified in the first two excluded opinions." (IB. 33.) But as set forth herein, Black's opinions as to the failure to monitor or intervene were based on pure speculation and quintessential "because I said so" opinions. They were properly excluded by the trial court.

Black's cause-of-death opinion is also admittedly contrary to the Medical Examiner's findings and determination of the cause of death, and is one she admits she is unqualified to make. *Race v. Bradford Cty.*, No. 3:18-cv-153-J-39PDB, 2020 U.S. Dist. LEXIS 248430, *9 (M.D. Fla. Dec. 15, 2020) ("Nurse Borelli is not qualified to testify regarding the cause of Ms. Seifried's death, as admitted in her deposition."); *Siegel*, 239 So. 3d at 743 ("Because Dr. Fisher's opinions are directly contradicted by the medical records upon which they are purportedly based, the trial court properly concluded that they were entitled to no weight."). As in *Race* and *Siegel*, Black's Third Opinion was properly excluded.

**CONCLUSION**

Specialty respectfully requests that this Court affirm the trial court's order excluding Black's opinions and the final summary judgment entered in its favor.

/s/ Carol M. Rooney
FL Bar No.: 72990
Hinshaw & Culbertson LLP
100 South Ashley Drive, Suite 1310
Tampa, FL 33602
Telephone: 813-868-8854
Facsimile: 813-436-8738
Primary: crooney@hinshawlaw.com
Secondary: jvaldez@hinshawlaw.com

/s/ Rory Eric Jurman
FL Bar No.: 194646
Hinshaw & Culbertson LLP
201 East Las Olas Boulevard, Suite 1450
Fort Lauderdale, FL 33301
Telephone: 954-467-7900
Facsimile: 954-467-1024
Primary: rjurman@hinshawlaw.com
Secondary: vharris@hinshawlaw.com; rejeserve@hinshawlaw.com
*Attorneys for Appellee*

## CERTIFICATE OF SERVICE

I certify that a copy of this filing has been provided by e-mail on the 16th of October 2026, to Douglas F. Eaton, Esq.; and William A. Dean, Esq., attorneys for Dorothy D. Jackson, as Personal Representative of the Estate of Ronnie L. Norris, Jr., Eaton & Wolk, P.L., 2665 South Bayshore Drive, Suite 609 Miami, Florida 33133, (305) 249-1640, deaton@eatonwolk.com, cgarcia@eatonwolk.com; Ford, Dean & Rotundo, P.A., 3323 NE 163rd Street, Suite 605, North Miami Beach, FL 33160, (305) 670-2000, Bill@forddean.com, Service@forddean.com.

/s/ Carol M. Rooney
FL Bar No.: 72990
Hinshaw & Culbertson LLP
100 South Ashley Drive, Suite 1310
Tampa, FL 33602
Telephone: 813-868-8854
Facsimile: 813-436-8738
Primary: crooney@hinshawlaw.com
Secondary: jvaldez@hinshawlaw.com

## CERTIFICATE OF COMPLIANCE

I certify that this document complies with the font requirements of Florida Rule of Appellate Procedure 9.045(b) (Arial 14-point) and with the word count limit of the applicable rule, and that it contains 10498 words, excluding the caption, cover page, table of contents, table of citations,

certificate of compliance, certificate of service and signature block, as Rule 9.045(e) provides.

/s/ Carol M. Rooney
FL Bar No.: 72990
Hinshaw & Culbertson LLP
100 South Ashley Drive, Suite 1310
Tampa, FL 33602
Telephone: 813-868-8854
Facsimile: 813-436-8738
Primary: crooney@hinshawlaw.com
Secondary: jvaldez@hinshawlaw.com